

CODE Correct public facing app — Concept Note

2026-01-20

Note

Purpose (discussion document): Develop a **public-facing, mobile-first demonstration version** of CODE Correct to **show feasibility**, build stakeholder awareness, and support eventual adoption—**without ID linkage**.

Estimated cost (order of magnitude): ~R100,000 (scope-dependent)

Non-negotiable: Proceed **only with National Department of Health (NDoH) endorsement** (“blessing”) and agreed governance conditions.

Background

A mobile medical cause of death certification (MCCD) app **CODE Correct** funded through the **Pandemic Preparedness Fund** (PI: Pam Groenewald) has been developed and plans to be piloted in 2026. CODE Correct has DORIS integration as well as ID number validation architecture. It also applies **South Africa-specific validation checks** to improve the quality of MCCD data at the point of certification. Specifically reducing ill-defined and unusable codes and reduce HIV/AIDS misclassification (increase the number of HIV/AIDS deaths reported)

However, much of the capability remains **invisible to many stakeholders and end-users**. A lightweight, public-facing version would make CODE Correct **tangible and demonstrable**, creating a practical platform for engagement, awareness-building, and future scaling discussions.

Why this activity

This is primarily a **demonstration and awareness initiative**, not a claim of immediate large-scale impact.

In this format, the app is **not expected to materially shift certification quality at scale on its own**. Its near-term value is to:

- **Demonstrate viability** and usability to NDoH and stakeholders
- Enable “show-and-tell” at workshops, governance forums, and training engagements
- **Create awareness among clinicians** regarding common quality issues in MCCD certification
- Serve as a key component of the **D4H-sponsored MCCD training campaign**, creating a **reciprocal referral loop** where the app directs users to training resources and the course promotes the app as a practical job aid
- Support NDoH and partners to **promote awareness** of CODE Correct and its intended role in improving data quality over time

Proposed activity (high level)

Develop a **public-facing CODE Correct “lite” demo app** (mobile web or similar) that provides:

- A small, representative set of **South Africa–specific validation checks and warnings**
- **Supportive prompts** illustrating how errors can be identified and corrected
- A simple interface designed for live demos and easy clinician interaction
- Data collection on common queries on the app as well as validation prompts that are suggested by users.
- A potential project is to link queries and validation checks to those who have and have not completed the D4H MCCD training course, a lighter version is for clinicians to self-report whether they have completed the MCCD course.

Out of scope for the demo app: patient identifiers, ID numbers, ID linkage, and any form of individual performance monitoring.

Intended use (primary)

- **Demonstration tool** for NDoH and stakeholders (meetings, workshops, roadshows)
- **Clinician engagement tool** to highlight quality issues and drive interest in best practices
- **Companion resource** for the D4H-sponsored MCCD training course
- **Awareness and uptake catalyst** alongside training and communications
- Practical artefact to support governance discussions on broader certification quality initiatives

Potential extensions (for discussion only)

Subject to NDoH preferences and governance approval, the demo app could later be extended to support:

- **Training reinforcement / MCCD course linkage:** from self-declared completion to verified linkage (higher governance + integration effort)
- **Campaign vehicle:** NDoH-led dissemination through professional bodies, facility channels, and training pathways (e.g., QR codes on job aids, circulars)
- **Micro-learning “nudges”:** short guidance snippets triggered by common validation issues
- **Community death job aid (cautious):** optional SOP-aligned checklists/prompts for DOA/community deaths—framed as consistency support, not a structural fix
- **Aggregate learning loop (non-identifying):** summarise common warning types to refine training and job aids (only if approved)

Guardrails (conditions of work)

This activity must proceed under explicit safeguards:

1. NDoH “blessing” required before build begins

- Written endorsement of the **concept** and agreement to **promote the tool**
- Named NDoH focal point(s) for coordination (not technical oversight)

2. Privacy-by-design (non-negotiable)

- **No patient identifiers** captured or stored
- **No ID linkage** in the public app
- Technical implementation ensures privacy compliance without requiring NDoH technical resources

3. No punitive monitoring

- No individual-level reporting to managers or regulators
- Any reporting limited to **aggregate, non-identifying summaries** (or none in MVP)

4. Scope discipline

- MVP is a **demonstration version**, not a full operational system
- Any expansion requires explicit governance sign-off and change control

Indicative outputs (MVP)

- Public-facing mobile demo app illustrating SA-specific validation prompts
- Optional: demo script / quick guide for stakeholder engagements

Cost and timeline (indicative)

- ~R100,000 - R200,000 (order of magnitude; depends on hosting/security and feature set)
- Rapid delivery of a standalone lite-app is feasible once governance and scope are agreed

Decision points (to unlock next steps)

1. Does NDoH endorse the development of a public-facing **demonstration** app as a concept?
2. Does NDoH agree to **promote** the app through agreed channels (e.g., circulars, training workshops, KnowledgeHub)?
3. Can we do minimal linkage of users to MCCD course completion and store data on common queries.

Note: Technical implementation, hosting, data security, and feature decisions will be managed entirely by the project team to minimize burden on NDoH.

Contacts / Owners (to complete):

- D4H lead: [Brian Brummer]
- NDoH focal point(s): [Tulani Mabose (Knowledge Hub), Mbhekiseni Khumalo, Milani Wolmarans]
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